NCCP 00448 · VERSION 4B

Panitumumab + FOLFIRI

Day 1: panitumumab, irinotecan, folinic acid and fluorouracil, followed by a fluorouracil pump for about 46 hours; disconnection is arranged around Day 3. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

Draft for clinical, pharmacy and patient/PPI review. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Eyes

Panitumumab

New eye pain or visual change needs prompt advice.

2 Mouth

Fluorouracil + folinic acid

Report ulcers or pain affecting eating and drinking.

3 Chest

Fluorouracil

New chest pain or breathlessness needs urgent assessment.

4 Bowel

Irinotecan / fluorouracil

Report diarrhoea, increased stoma output or severe abdominal pain.

5 Hydration / minerals

Panitumumab + chemotherapy

Reduced urine, cramps or weakness need advice; keep your blood-test appointments.

6 Skin / nails

Panitumumab

Report rash, cracks or painful nails early.

7 Hands / feet

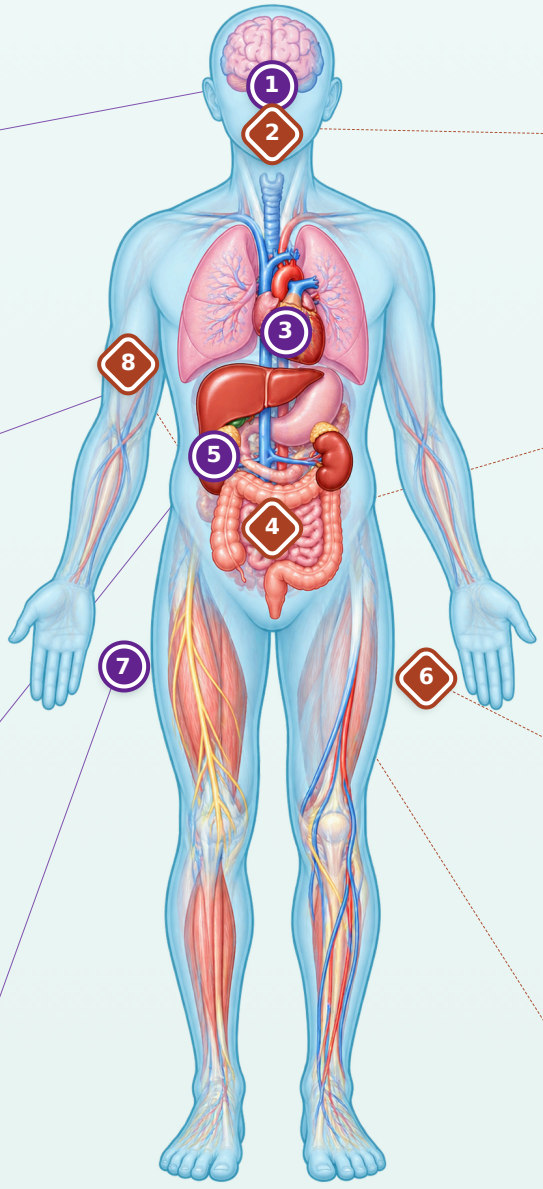
Fluorouracil

Report painful redness, peeling or difficulty using your hands or walking.

8 Pump / line

Treatment delivery

Pain, leakage, swelling or a loose connection needs immediate advice.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

NCCP 00448 · PRACTICAL SUPPORT

Your treatment, at home.

Important medicine-specific effects

Panitumumab

- Rash, dry skin, cracks and sore nails
- Eye inflammation or corneal injury
- Low magnesium, calcium or potassium
- Diarrhoea, dehydration and kidney problems
- Infusion reaction; rare serious lung or skin effects

Irinotecan

- Early cramps, sweating and diarrhoea
- Delayed diarrhoea and dehydration
- Low blood counts / serious infection
- Nausea, mouth soreness and fatigue
- Hair thinning or loss
- Rare serious lung or allergic reactions

Folinic acid

- Enhances fluorouracil activity and toxicity
- Rare allergic reaction
- Diarrhoea / mouth soreness in combination

Fluorouracil

- Diarrhoea, dehydration and mouth ulcers
- Low blood counts / infection
- Hand-foot skin soreness
- Cardiotoxicity including chest pain / ischaemia
- Severe early toxicity including DPD-related toxicity
- Pump / central-line complications

Make the plan practical

Start skin care early

Ask for your skin-care plan before the first infusion. Use the recommended moisturiser and sun protection; your team may prescribe a cream or an antibiotic. Do not use ordinary acne treatments without advice.

Minerals and blood tests

Panitumumab can lower magnesium and other minerals even when you feel well. Blood tests continue during treatment and for about eight weeks after it finishes. Take replacement treatment only as prescribed.

DPD testing before fluorouracil

DPD helps the body break down fluorouracil. Deficiency can cause severe toxicity. Testing guides the plan but cannot exclude all serious reactions; unexpectedly severe early diarrhoea, mouth sores or illness needs immediate review.

Your diarrhoea plan

Have the prescribed anti-diarrhoeal medicine and written instructions before going home. Follow that plan as soon as diarrhoea starts. Call promptly for persistent symptoms, cramps, fever, blood in stools or reduced urine.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

CUH · MON–FRI 8AM–5PM

087 181 9645

Acute Oncology Service

CUH · WEEKENDS / BANK HOLIDAYS 9AM–5PM

021 4922000

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00448/

Sources and review status are available online. Follow your own prescription and hospital instructions.